Intro to Health Assessment

NURA 330 Health Assessment | Module 1 Lecture Study Guide

Exam 1 content · reviewed for the final

Lecture roadmap: History of health assessment → RN responsibilities → Clinical judgment (recognize and analyze cues) → Purpose and definition of health assessment → Health history and subjective data → Objective data; comprehensive vs. focused assessment → Synthesizing cues → Trauma-informed context → The four techniques: inspection, palpation, percussion, auscultation → Case study

Exam 1 blueprint: Module 1 was worth **10–12 questions** (of 50). The study guide focused on: purpose and definition of health assessment; nursing process and clinical reasoning; subjective vs. objective data (including primary vs. secondary sources); head-to-toe vs. focused assessment; and the four physical assessment techniques.

How to read this guide: Everything without a tag comes from the lecture slides or the slide notes. Anything else is tagged: STUDY GUIDE emphasized on the exam study guide but not spelled out on the slides; ADDED information not in the slides; CLARIFIED a slide point explained further; CORRECTED an error or inconsistency in the slides. Every tag is also listed in the notes at the end.

Why Health Assessment Matters

A patient tells you things they might not tell anyone else, then undresses and lets you touch them. The lecture opens with Dr. Abraham Verghese's point that our exam skills have to be **worthy of that kind of trust**.

- **History:** Medicine has used assessment techniques for thousands of years. In the late 1800s, Florence Nightingale tracked environmental factors (ventilation, light, hygiene) to promote healing. Health assessment entered undergraduate nursing curricula in the **1970s**. Laennec invented the stethoscope in 1816.
- **Today's emphasis is holistic:** a complete picture of the patient's biological, psychological, and social needs. Nursing education has also shifted away from advanced techniques (e.g., diaphragmatic excursion) toward **clinical judgment**.

Key Responsibilities of RNs (ANA)

1. **Perform physical exams and health histories before making critical decisions**, the focus of this course.
2. Provide health promotion, counseling, and education.

3. Administer medications and other personalized interventions.
4. Coordinate care with a wide range of healthcare professionals.

Clinical Judgment

Definition (AACN): Clinical judgment is "the process by which nurses make decisions based on nursing knowledge (evidence, theories, ways/patterns of knowing), other disciplinary knowledge, critical thinking, and clinical reasoning."

In the NCSBN Clinical Judgment Measurement Model, **recognizing cues** and **analyzing cues** are the first steps. You cannot prioritize problems, take safe action, or evaluate outcomes without an accurate baseline of subjective and objective data. That is why **complete, accurate assessment data is the foundation for all later care**.

Course	Clinical judgment step it trains
Health Assessment (NURA 330)	**Recognize cues** (taking a structured history, doing a systematic exam) and **analyze cues** (telling normal findings from abnormal ones)
Pathophysiology (NURA 340)	The science behind **analyzing cues**: *why* the patient has these findings
Pharmacology	The science to **generate solutions** once abnormal cues are recognized
Fundamentals	Skills to **take action** safely (infection control, medication "rights")

ADDED **The full model:** The NCSBN model has six steps: **recognize cues → analyze cues → prioritize hypotheses → generate solutions → take action → evaluate outcomes**. The slide shows the model as an image; the step names are listed here so you can see where assessment fits.

Purpose and Definition of Health Assessment

Definition (ATI): Health assessment "involves the collection, clinical judgment, and evaluation of data to plan and deliver patient-centered care while accounting for the client's preferences, goals, and needs." Data include **physiological, psychological, sociocultural, spiritual, economic, and lifestyle** factors.

Purpose	What it means
Establish a baseline	Gather historical and physical data to compare against later
Database creation	Combine subjective and objective data into a holistic patient profile
Clinical decision-making and diagnostic reasoning	Form nursing diagnoses, identify actual or potential problems, set priorities
Personalized care planning	Patient-centered interventions, measurable outcomes, and plans tailored to physiological, psychological, and sociocultural needs
Evaluate care effectiveness	Reassess after interventions to confirm positive outcomes
Health promotion and prevention	Identify risk factors, habits, and education needs to promote wellness and prevent illness

Assessment takes clinical judgment beyond technical skill: deciding **when, what, and how** to evaluate a patient.

Subjective vs. Objective Data

A health assessment usually has **two parts**: a **health history** (subjective) and either a **comprehensive** or **focused** physical assessment (objective).

	Subjective Data (Symptom)	Objective Data (Sign)
What it is	The patient's own perspective, feelings, perceptions, and lived experience; **reported** by the patient, not observed or measured by the nurse	**Measurable and observable** findings gathered through sight, hearing, touch, and smell, plus equipment
Source	**Primary:** the patient. **Secondary:** care partners (family, caregivers) STUDY GUIDE	The nurse's senses and diagnostic tools
Examples	Pain intensity, health history, SDOH, "I feel dizzy"	Vital signs, pupil response, edema, breath sounds
Collected during	Health history interview	Physical exam

CLARIFIED **Primary vs. secondary sources:** The slide labels the patient "primary" and care partners "secondary." Records, other clinicians, and lab results are also commonly counted as secondary sources. When a family member answers for the patient, the information is still subjective (it is someone's report), but it comes from a secondary source, so note who gave it.

Synthesizing cues: Neither type of data stands alone. Subjective complaints (recognizing cues) are checked and compared against the physical exam (analyzing cues). Combining them lets the nurse prioritize, choose safe interventions, and evaluate outcomes.

The Health History (Subjective)

A structured, client-centered interview that systematically collects subjective data using culturally responsive, developmentally appropriate communication. Main parts (covered in detail in Module 2):

- **Demographics and context:** identity, gender and pronouns, preferred language
- **Chief concern and History of Present Illness (HPI):** the main reason for seeking care
- **Past medical history:** prior conditions, treatments, risk factors
- **Social history and SDOH:** daily life and environmental factors
- **Review of systems (ROS):** a head-to-toe checklist of symptoms the patient reports, to catch problems not mentioned in the HPI

Comprehensive vs. Focused Assessment (Objective)

	Comprehensive (Head-to-Toe)	Focused
What	Systematic exam of **all** body systems	Targets **one system or area** based on the chief complaint
Where / when	Stable settings: primary care visits, long-term care admissions, hospital admissions	EDs, urgent care, inpatient units when a problem arises
Goal	Establish a holistic baseline	Identify and prioritize immediate, time-sensitive concerns
Example	Annual physical	A patient gasping for air in the ER: assess the respiratory system, not childhood vaccines

Contextual Awareness: Trauma-Informed Care

The lecture frames assessment within a complex U.S. healthcare system:

- The U.S. has the highest rate of multiple chronic conditions, the lowest life expectancy, and the highest preventable mortality of high-income countries.
- About **51.8%** of U.S. adults have at least one chronic condition, and **27.2%** have multiple.
- Demand for care is outpacing the supply of providers, and nurses are often overwhelmed.
- **The problem:** without a trauma-informed approach, patient-centered care suffers, outcomes worsen, and trust in the healthcare system declines.
- **Human complexity:** poor health has multiple causes, including physical and behavioral health, structural and social conditions, racism, stigma, and bias. Effective care requires empathy for that complexity.

The Four Physical Assessment Techniques

Order matters: The usual order is **Inspection → Palpation → Percussion → Auscultation**. **Exception: the abdomen** is **Inspect → Auscultate → Percuss → Palpate**, because pressing on the abdomen can stimulate bowel activity and change the bowel sounds.

1. Inspection

Observation of the patient, including general appearance. It is the "clinical detective's" first step: eyes, ears, and nose tell a story before you touch the patient.

Level	What you do
General survey (macro view)	Whole-person behavior and immediate distress: age, pronouns, alertness, hygiene, posture, nonverbal distress, affect and mood. Goal: a rapid impression of whether the patient is in immediate danger
Systematic visual scan (micro view)	Compare right and left sides for symmetry; skin color (pallor, jaundice, cyanosis), rashes, bruises, wounds, incisions; breathing rate, effort, and chest rise
Multisensory inspection	**Hearing:** audible wheezing or gasping. **Smell:** sweet or fruity breath (a warning sign of diabetic ketoacidosis), foul wound odor (possible infection)
Toolkit	Hand hygiene, adequate lighting, penlight (PERRLA), paper tape measure (wounds)

Protect dignity, privacy, modesty, and comfort. **Always explain what you are examining and why before you do it.**

2. Palpation

Systematic, purposeful touch to assess temperature, moisture, texture, and underlying structures: the "tactical touch-map" that confirms what your eyes suspected (e.g., you see leg swelling, then palpate to see whether it pits or is tender).

Hand surface	Best for	Why
Dorsal surface (back of hand)	**Temperature** (warm, feverish, cold, clammy)	Thinner skin, so more sensitive to temperature
Palmar finger pads	**Fine discrimination:** texture, moisture, masses, **pulses**	Highest density of tactile nerve endings

Trauma-informed touch: being touched by a stranger can cause anxiety and muscle guarding. Build trust, get consent, and perform hand hygiene first. *Tissue compliance* = how yielding, elastic, or resistant tissue feels under pressure.

ADDED **One more surface often tested:** The **ulnar (side) surface of the hand** is most sensitive to **vibration** (e.g., tactile fremitus). It is not on the slide but appears in many nursing references.

3. Percussion

Tapping the body surface to produce vibrations that reveal the **density** of tissue underneath. Used to map organ size and borders, or to detect abnormal air, fluid, or masses. **Not widely used by novice nurses.**

Sound	Quality	Normally heard over
Resonance	Clear, hollow	Air-filled lungs
Tympany	High-pitched, drum-like	Air-filled stomach or bowel
Dullness	Soft, deadened thud	Solid organs (liver). **Abnormal over the lungs** (fluid or mass)

4. Auscultation

Listening to cardiovascular, respiratory, and GI sounds with a stethoscope.

- Angle the **earpieces forward (toward your nose)** so sound goes straight to the eardrum.

- Place the chest piece **directly on the skin**, not over clothing.

Chest piece	Detects	Examples
Diaphragm	**High-pitched** sounds	Lung sounds, bowel sounds, normal heart sounds (S1, S2)
Bell	**Low-pitched** sounds	Murmurs, bruits, extra heart sounds (S3, S4)

ADDED **Technique tip:** Press the diaphragm **firmly** against the skin; rest the bell **lightly**. Pressing the bell hard stretches the skin into a diaphragm and filters out the low sounds you are trying to hear.

Case Study: James Walker

58-year-old African American male, routine check-up. "I've been feeling really fatigued lately, and my feet have been swelling up by the end of the day."

Subjective (health history)	Objective (physical exam)
Chief concern (in his words, above) **HPI:** bilateral lower extremity swelling × 3 weeks; daily exhaustion **PMH:** hypertension (5 yrs), type 2 diabetes (3 yrs) **SDOH:** takes meds sporadically because of copays; lives in a food desert	**VS:** BP 148/92, HR 78, RR 18, T 98.4°F, BMI 31.2 **Inspection:** bilateral +2 pitting edema to mid-shin **Palpation:** skin warm (dorsal surface); +2 dorsalis pedis pulses (finger pads) **Auscultation:** S1/S2 present, no murmurs; lungs clear bilaterally

Discussion: His long-standing, inconsistently treated hypertension and diabetes help explain the elevated BP and bilateral edema. The SDOH findings (cost, food access) are part of the cause, and part of the plan.

Quick Review

Concept	Key points
Health assessment	Collection, clinical judgment, and evaluation of data to plan patient-centered care
Clinical judgment	Assessment = recognize cues and analyze cues (the first steps)
Subjective	Symptom; patient's report; primary = patient, secondary = care partners
Objective	Sign; measurable/observable via senses and equipment
Comprehensive vs. focused	All systems for a baseline vs. one system for an urgent problem
Technique order	I → P → P → A; abdomen: I → A → P → P
Hand surfaces	Back of hand = temperature; finger pads = texture, moisture, masses, pulses
Percussion sounds	Resonance = lungs; tympany = stomach/bowel; dullness = solid organs (abnormal over lungs)
Stethoscope	Earpieces toward the nose; on bare skin; diaphragm = high pitch; bell = low pitch

Self-Check ADDED

1. A patient's daughter reports that her mother "hasn't been eating for days." Is this subjective or objective data, and what is the source? *Answer: Subjective, from a secondary source.*
2. Which hand surface should the nurse use to assess skin temperature? *Answer: The dorsal surface (back of the hand).*
3. In what order are techniques performed for the abdomen? *Answer: Inspection, auscultation, percussion, palpation.*
4. A patient arrives in the ED with severe shortness of breath. Comprehensive or focused assessment? *Answer: Focused (respiratory).*
5. Dullness heard on percussion over the lungs suggests what? *Answer: Fluid or a solid mass where air should be.*

Notes on Sources, Additions & Clarifications

Corrections:

- The slide notes say the four techniques are "Inspection, Palpation, Percussion, and Palpation"; the last should be **auscultation**. The order the notes then give is correct.

From the Exam 1 study guide (not spelled out on the slides):

- The question count for Module 1 (10–12).
- "Primary versus secondary origins" of subjective data. The slides list the patient as primary and care partners as secondary; the study guide asks you to know this distinction.

Added information (not in the slides):

- The six steps of the NCSBN Clinical Judgment Measurement Model (the slide shows it only as an image).
- Ulnar surface of the hand for vibration.
- Diaphragm pressed firmly, bell rested lightly.
- The self-check questions.

Clarified:

- Other common secondary sources (records, other clinicians, labs), and that a caregiver's report is still subjective data.

Not included: the slides' reference list and the image-only slides (chronic disease map, stock photos). Their text is summarized in the trauma-informed care section.